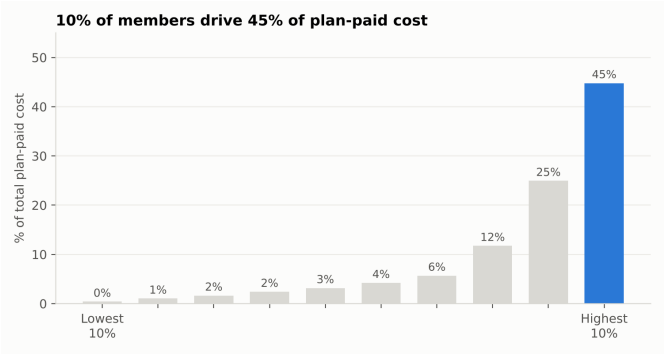
What Our Data Says About Cost and Care

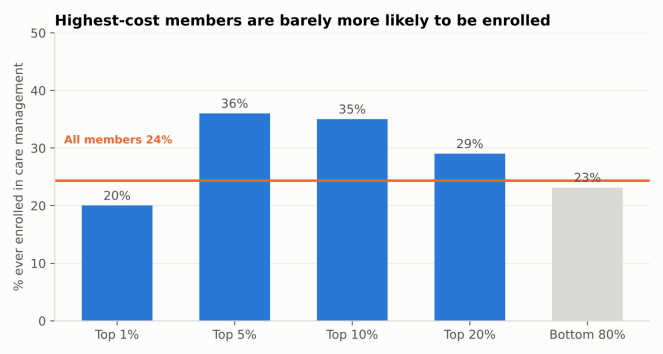
A plain-English summary for Frostmore Stormrock and the Board

I looked at claims, hospital and ER visits, care management enrollment and prior-authorization decisions for about 1,000 of our ~250,000 members, from January 1 to October 15, 2025. Because it is a sample, please read the percentages and patterns, not the dollar totals. Here is what I found, what I think it means, and what I would do next.

The short answer

- **A small group of members drives a big share of our costs.** About 1 in 10 members accounts for **45 cents of every dollar** we pay, and hospital stays make up about **69 cents of every dollar**.
- **Our care management programs are not aimed at those members yet.** Of our 50 most expensive members, only about 36% are enrolled, barely better than the 24% across everyone.
- **We cannot yet say whether the programs work.** I found no clear drop in ER visits or hospital stays after members enrolled, but the data is too thin to settle the question either way.





Left: each bar is 10% of members, from lowest cost to highest. Right: the share of each group that has ever been in care management.

What I found

- **The people who need the most help are easy to spot.** I scored members on cost, hospital stays, ER visits and long-term conditions. **73 members (about 7%) stand out as high risk** and account for 26% of spending. **37 of them have never been in care management.** I have listed them by member ID in a separate outreach file.
- **Past cost did not predict future cost.** When I tested the score on the first half of 2025, it barely predicted the second half. High costs here come mostly from one-time hospital stays, not the same people every month. So we should reach out when something happens (a hospital discharge, repeat ER visits), not only by looking at last year's bill.
- **Many care management spots are tied up.** 187 of our 244 active enrollments are more than six months old, and 65 of those never got a care plan. Only 35% of enrollments reach even half of their goals.
- **One pocket of hospital spending deserves a second look.** 83 hospital stays, costing $1.60M (**18% of everything we paid**), were billed with diagnoses like a routine check-up, a cold or a headache. These rarely need a hospital bed. It may be a billing or coding problem, or stays that were not needed. A small review would tell us which.
- **Heart conditions cost the most among chronic illnesses**, about $11,262 per member, roughly 28% more than other members. Diabetes and high blood pressure cost less than average.
- **Approvals and denials need attention.** We deny about 21% of prior-authorization requests, and when providers appeal, **41% of decisions are reversed**, a sign that some first answers do not hold up. Urgent requests take almost 4 days on average. Providers also see 394 claim denials, most often for missing prior authorization.

What I recommend for 2026

1. Review the questionable hospital stays. Have clinicians and coders audit a sample of the 83 stays. This is the biggest single saving I found.

2. Point care management at the right people. Start with the 46 high-risk members not currently enrolled, and enroll people after a hospital stay or repeated ER visits.

3. Free up spots. Review the 187 older enrollments, close the ones that are finished or inactive, and make sure completion is recorded the same way everywhere.

4. Measure results properly. Add last year's claims and member enrollment records, and compare enrolled members with similar members who are not enrolled. Then we can say with confidence whether programs pay off.

5. Make approvals smoother. Teach providers the prior-authorization rules, check why so many first denials are reversed, and make urgent requests faster. Also, some reports we use do not match our records (for example, prior-authorization time is reported as 2.5 days but our data shows about 4), so let's agree on definitions.

How sure am I?

Very sure: costs are concentrated in a few members and in hospital stays, and care management is not reaching the highest-cost members. **Fairly sure:** the questionable hospital stays are worth a review (the diagnoses could be a data quirk). **Not sure yet:** whether the programs reduce cost or hospital use. Only 68 members had enough before-and-after data, so I would not claim savings or harm. This sample also has more hospital and ER use than a typical health plan, so it may lean toward heavy users.

Quick glossary

Care management: nurses or coordinators who help members with serious or complex health needs. **Inpatient:** an overnight hospital stay. **ER / ED:** emergency room. **Prior authorization:** the plan's approval before certain services. **High-risk:** in this report, members with high cost, hospital or ER use, or several long-term conditions.